

Unlocking the science of longevity to develop transformative therapies

Corporate Presentation • April 2026

Forward-looking statements

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Overview

Our lead asset bezisterim (formerly NE3107) modulates the production of TNF α . In clinical trials, many patients treated with bezisterim experienced:

- Reduced inflammation and the associated insulin resistance
- Improved motor control and “morning on” symptoms in Parkinson’s disease (PD)
- Improved cognition and function, lowered amyloid β and p-tau levels, and improved brain imaging scans in Alzheimer’s Disease (AD)
- Lowered DNA methylation levels

BIV201 reduces fluid build up and has the potential to become the first therapeutic for ascites, a condition with 50% mortality rate within 12 months

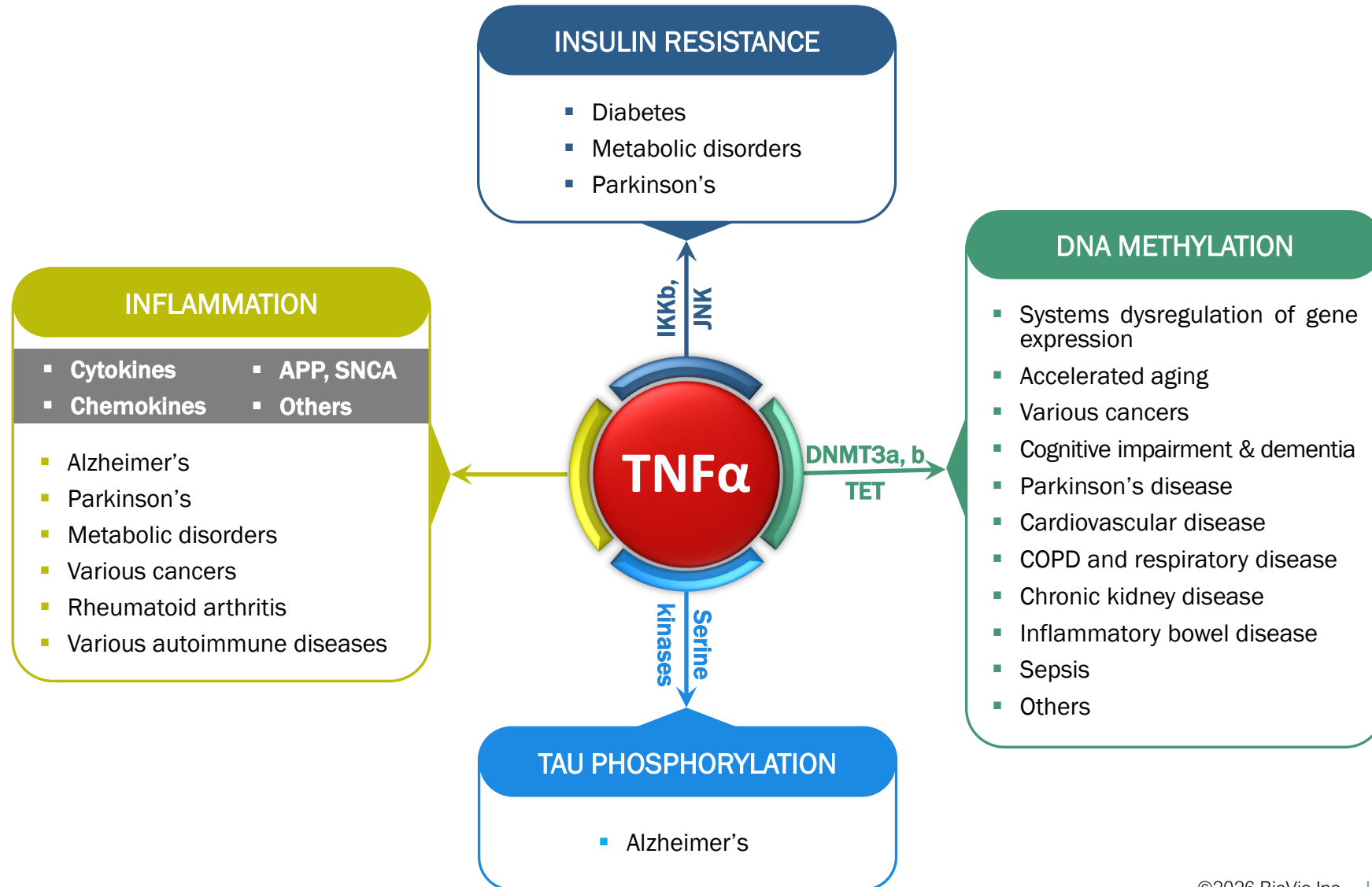
Our priorities:

- Complete Parkinson’s Phase 2b trial – topline data 2Q26
- Complete Long Covid exploratory Phase 2 – topline data Summer ’26
- Launch ascites Phase 3 when funding becomes available
- Launch next Alzheimer’s Phase 3 trial using new once-daily formulation of bezisterim upon securing funding

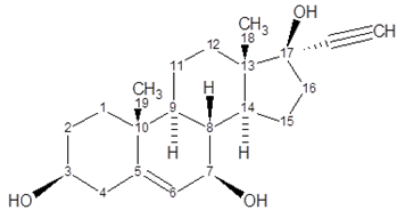
Near-term Catalysts

- Topline readout from Parkinson's Trial – 2Q26
- Topline readout from Long Covid Trial – Summer '26

Far-reaching impact of TNF α -mediated chronic low-grade inflammation



Bezisterim's mechanism of action

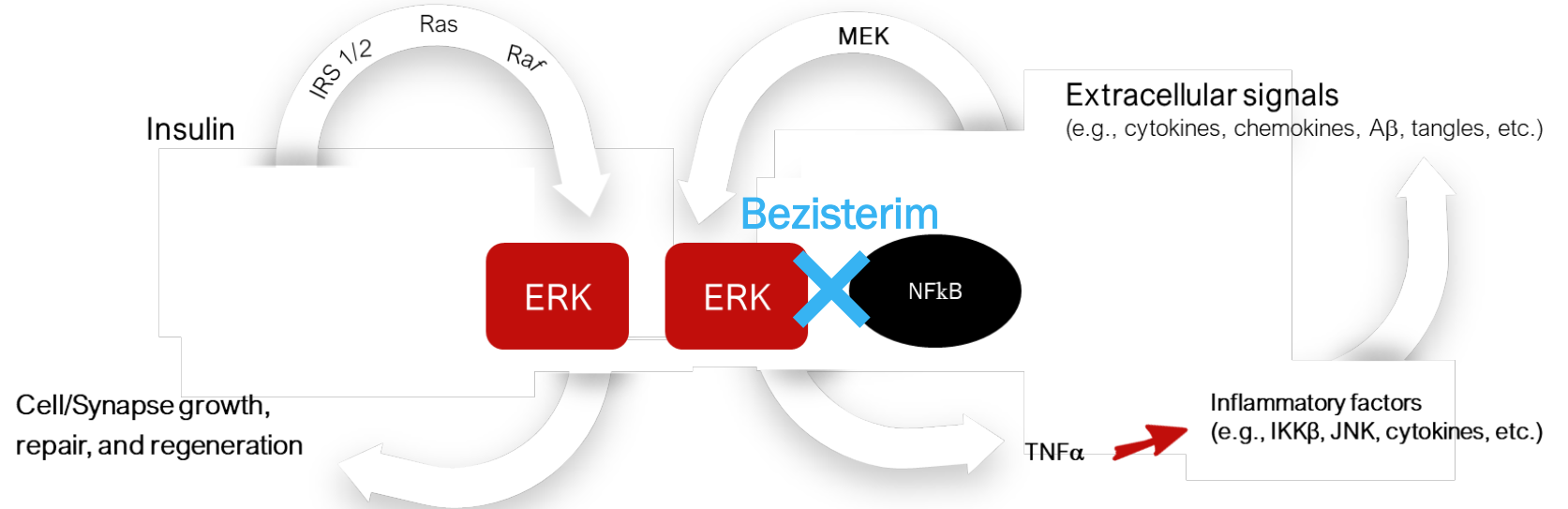


First-in-class molecule with desirable characteristics

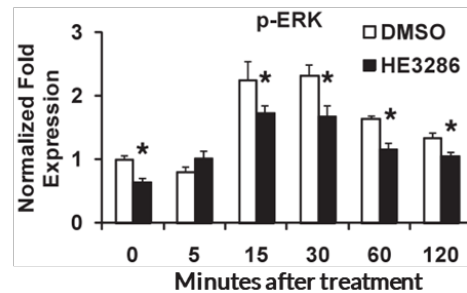
Small molecule; orally bioavailable

Crosses blood-brain barrier, thus CNS and peripheral applications

No safety issues identified to date in pre-clinical and clinical trials (up to Phase 2)

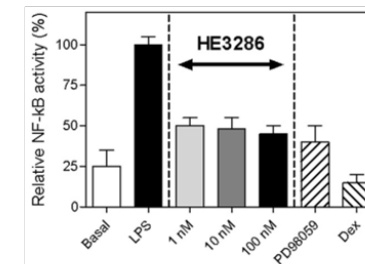


Bezisterim Reduces ERK Activation



Lu 2010 Am J Physiol Endocrinol Metab 298 E1036

Bezisterim Reduces NFκB Activation



Wang 2010 J Pharmacol Exp Ther 333 70

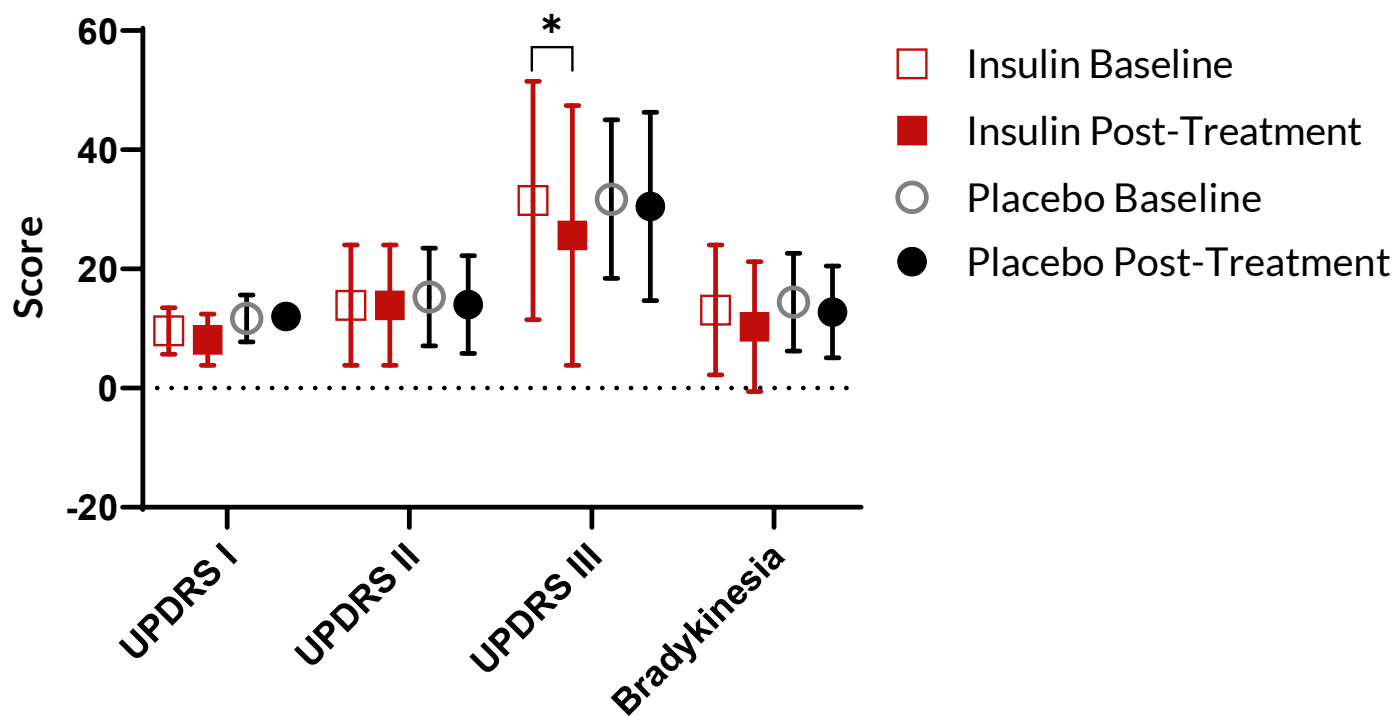
Bezisterim in Parkinson's Disease

Insulin resistance drives neurodegeneration and neuron dysfunction

Impact Domain	Role of Insulin Resistance (IR)
Insulin signaling in the brain	IR impairs the activation of Protein Kinase B (PKB/Akt), leading to reduced glucose metabolism and impaired metabolic control, neuronal survival, and synaptic plasticity
Dopamine regulation	- IR impairs the expression of Tyrosine Hydroxylase, an enzyme essential for making dopamine - IR impairs the function of dopamine transporters, thereby compromising the brain's ability to release and take up dopamine
α -synuclein accumulation	- IR impairs insulin-degrading enzyme (IDE) function, reducing clearance of misfolded proteins and promoting α-synuclein aggregation into toxic Lewy bodies - IR increases GSK-3β activity, enhancing α-synuclein phosphorylation and making it more toxic and resistant to degradation
Mitochondrial dysfunction & oxidative stress	- IR disrupts the Parkin/PINK1 pathway responsible for clearing damaged mitochondria, leading to accumulation of dysfunctional mitochondria and increased release of reactive oxygen species (ROS) - The resulting oxidative stress damages dopaminergic neurons in the <i>substantia nigra</i> - IR reduces PGC-1 activity, a key regulator of cellular energy production, effectively depriving neurons of the ATP needed for proper function.

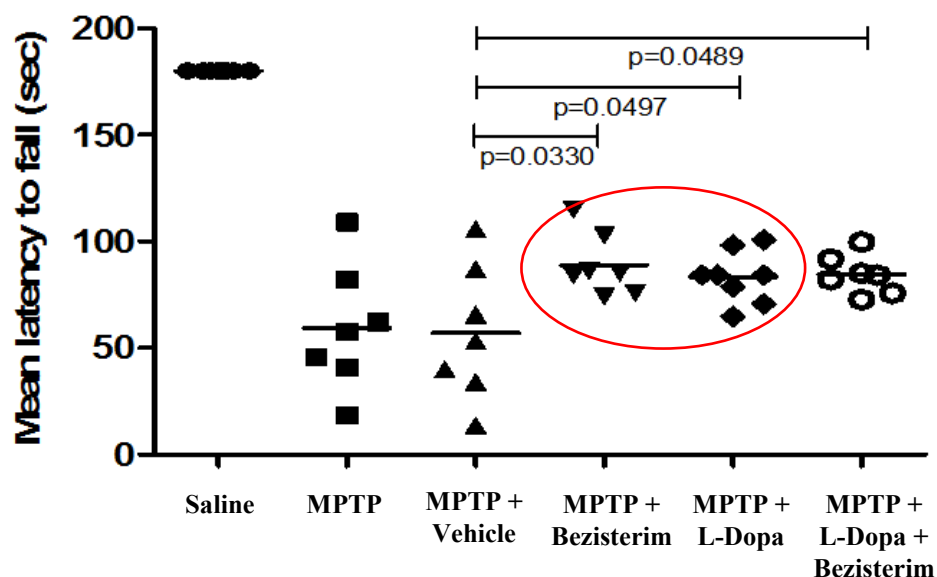
Low Dopamine *and* inflammation are required to develop Parkinson symptoms in patients

Intranasal Insulin treatment reduced inflammation and improved motor activity

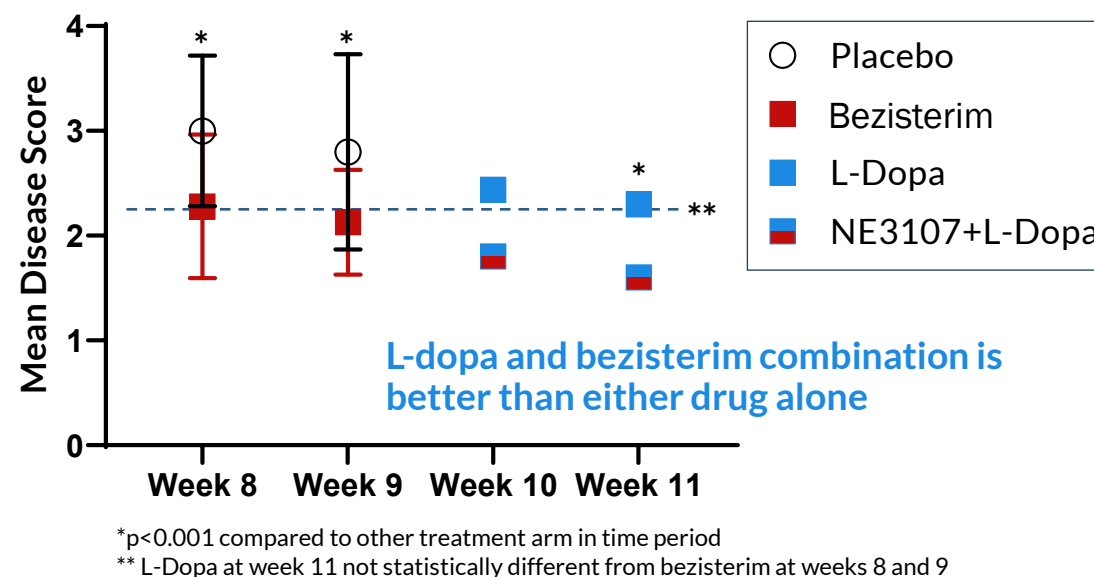


Bezisterim has similar promotoric activity to L-dopa in rodent* and marmoset* models

MPTP Mouse



MPTP Marmoset treated at Week 8

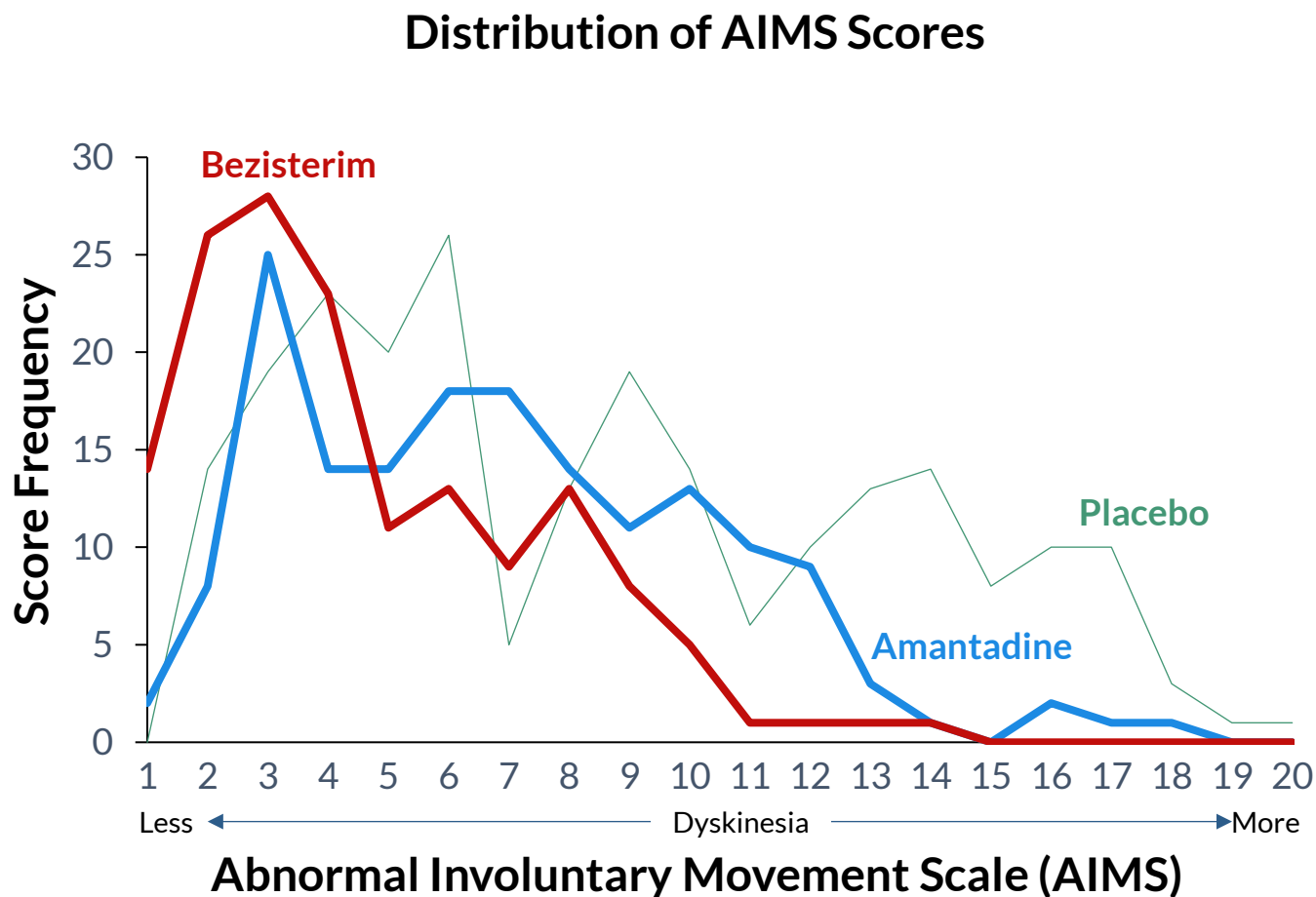


bezisterim's promotoric effects observed within 4 days of treatment

Bezisterim decreases L-Dopa-induced dyskinesia (LID) in marmosets*

Funded by

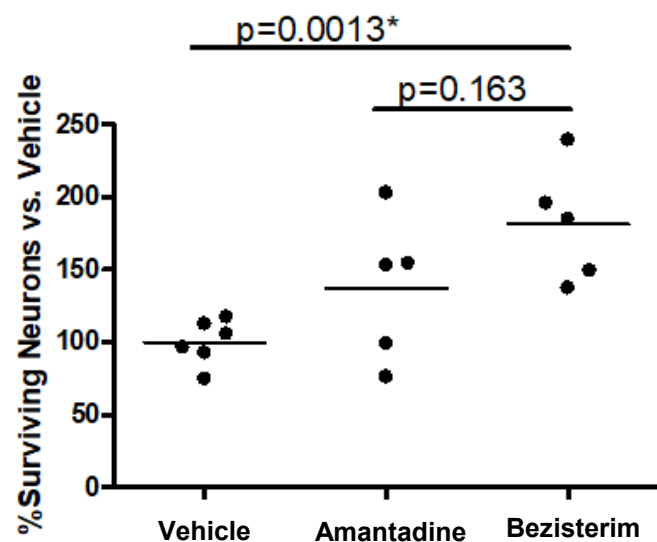
THE MICHAEL J. FOX FOUNDATION
FOR PARKINSON'S RESEARCH



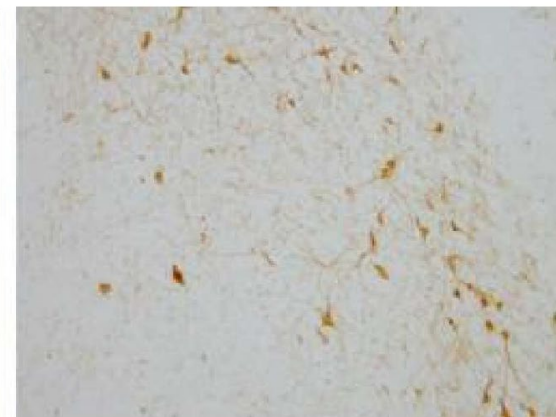
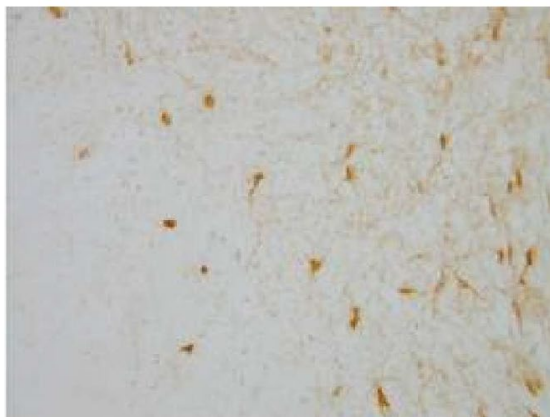
Bezisterim preserved ~2X TH+ (dopaminergic) neurons in MPTP marmosets*

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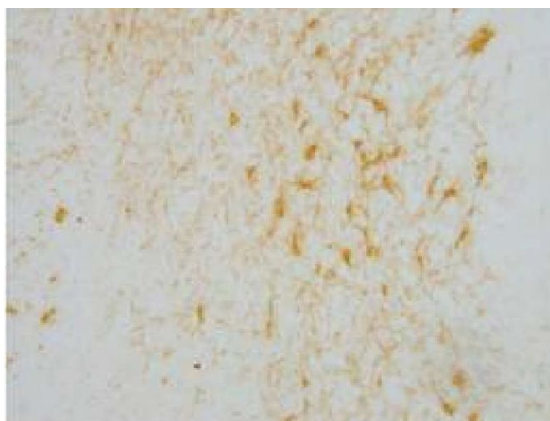
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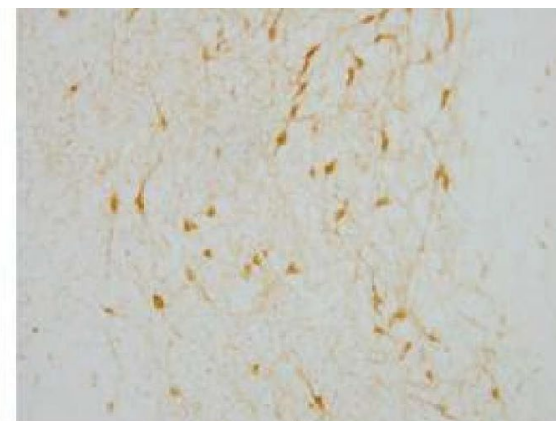
Control moneys (M09100 and M11008)



Bezisterim



Amantadine (M10084)



Parkinson's Disease Clinical Development Program

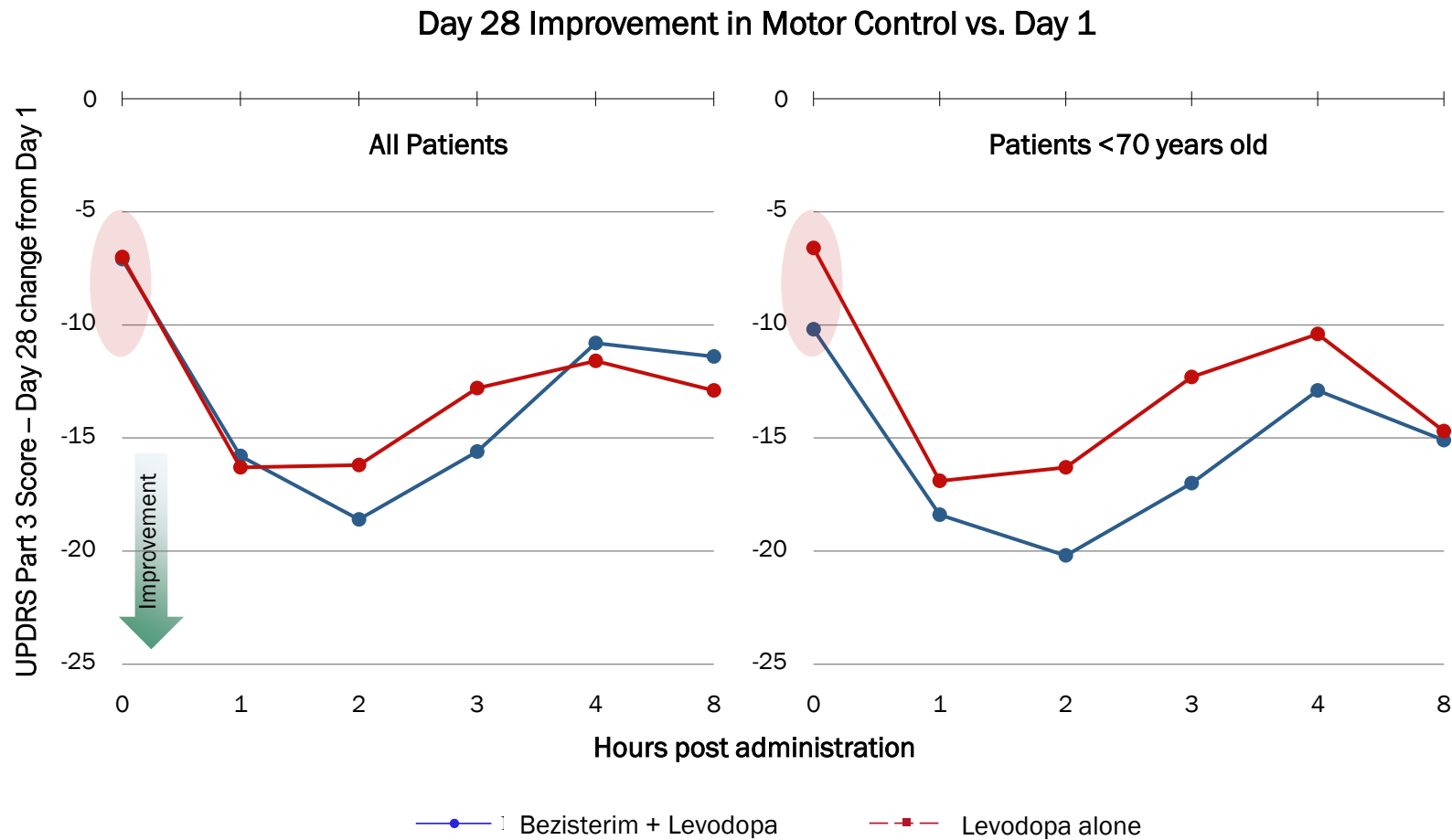
NM201 Phase 2

- Satisfying FDA requirement for drug-drug interaction study with L-dopa
- Detect efficacy signal for bezisterim's pro-motoric activity

40 patients with defined L-dopa "off state", 1:1 active: placebo, 20 mg BID for 28 days

- **Safety assessments:** Standard measures of patient health, L-dopa PK and DDI
- **Efficacy assessments:** MDS-UPDRS* parts 1-3, Hauser ON/OFF Diary, Non-Motor Symptom Scale

Bezisterim-treatment patients experienced fewer motor symptoms before morning drug administration

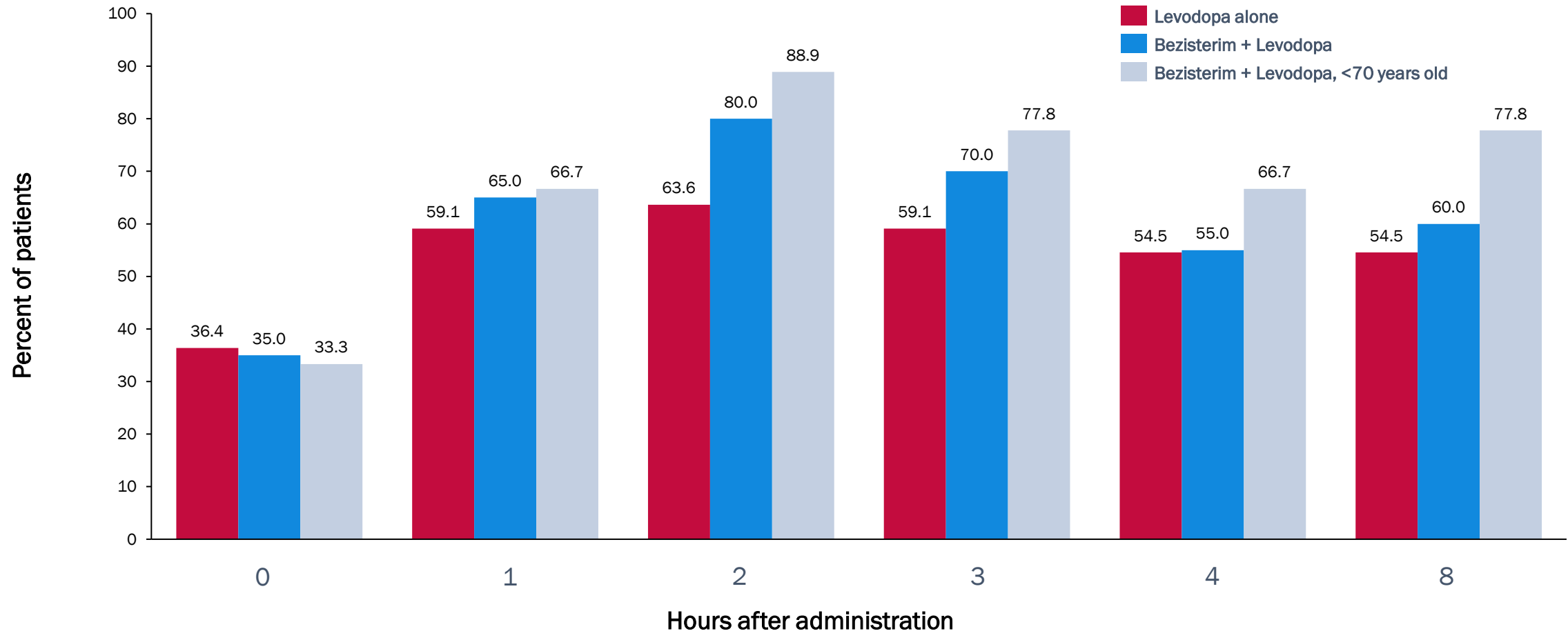


	Bezisterim	Placebo
"On" at t=0	6	0
Total patients	20	19
P-value*	0.02	

* Fisher's exact test

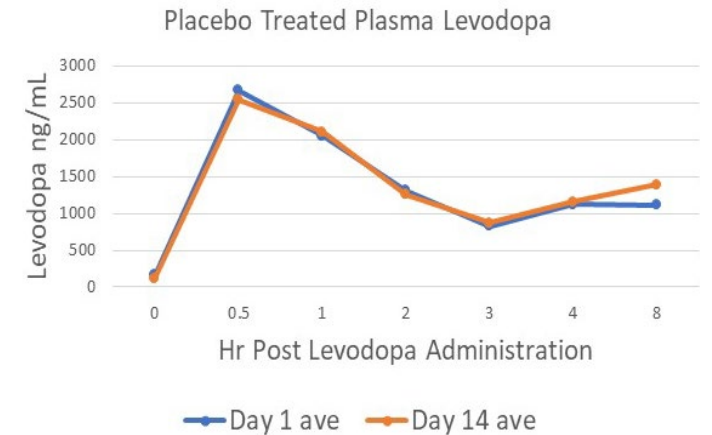
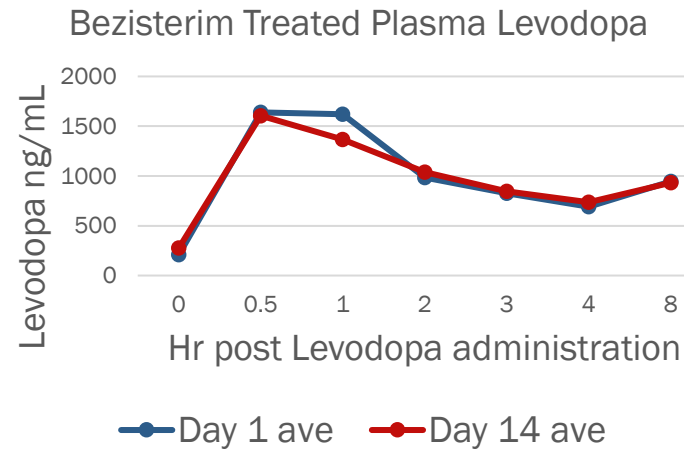
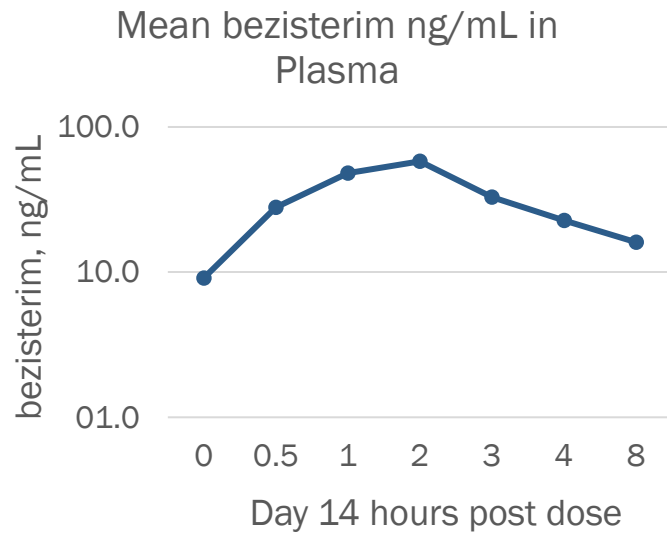
Larger proportion of patients treated with bezisterim had >30% improvements in motor control

Percentage of patients experiencing >30% improvement at Day 28 vs. Day 0



Desirable pharmacokinetics – no observed DDI

0-8 hr 231 ngh/mL (SD 22.38)



PD Clinical Development Plan

	NE3107-PD-202 (FPFV 1Q25)	NE3107-PD-301 (conceptual)
Objective	• Demonstrate motor and non-motor impact as monotherapy treatment • Forecast potential impact on disease progression with DNA methylation changes	Demonstrate QOL, motoric and disease progression benefits
Phase	2	3
N, 1:1 vs placebo	60	300-400
Patients	Treatment naïve, Part 3 $\geq$ 12	Diagnosis within past 5 y
Treatment duration	12 weeks	26-52 weeks + 52 weeks open label
Dose	20 mg BID	20 mg BID

Bezisterim in Long COVID

Long COVID

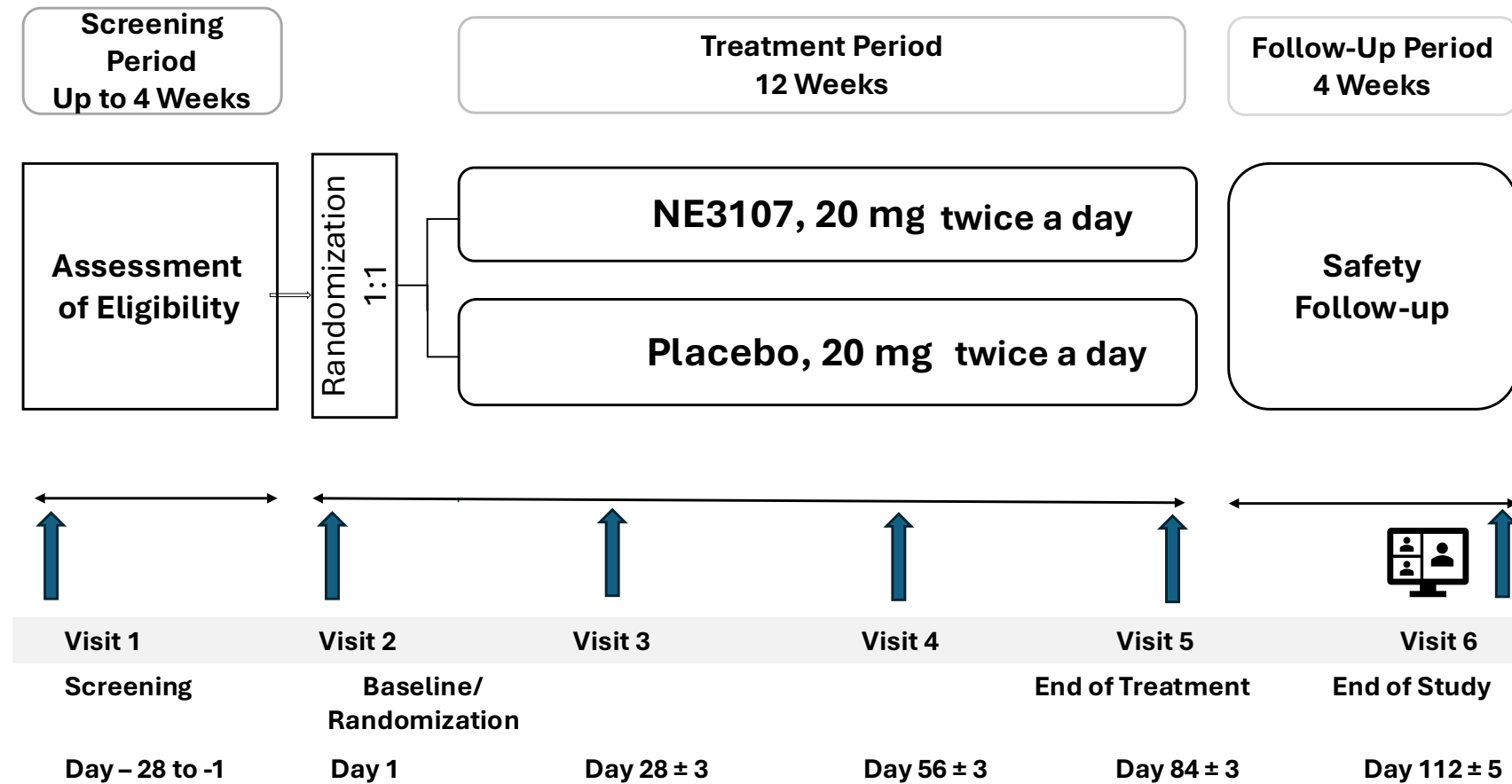
- Persistent symptoms of at least 3 months following SARS-CoV-2 infection
- Most common symptoms brain fog, fatigue and post-exertional malaise
- Currently 17 million adults in US suffering from Long COVID
- Despite decreasing incidence due to milder SARS-CoV-2 variants and vaccination, the incidence rate of LC is still ~ 5% which translates to ~30,000 new patients/day
- No approved treatments – large unmet need

Why study Bezisterim in Long COVID?

- Sustained inflammation through TLR4 / RAGE loop leads to symptoms of Long COVID
 - Spike protein persists in circulation after COVID and can cross the BBB
 - Spike protein induces cognitive dysfunction via TLR4 activation
 - TLR variant associated with Long COVID cognitive dysfunction and severe Long COVID
- The envelope proteins (E proteins) and Spike protein of SARS-CoV-2 signal through TLR2 and TLR4 and subsequently activate nuclear factor kappa B (NF- κ B) and extracellular signal-regulated kinase (ERK)
- Bezisterim has been shown to modulate the activation of ERK and NF- κ B and thus should impact Long COVID symptoms

NE3107-LC-201 Trial Design:

Double-blind, placebo-controlled, 12-week treatment period



Trial Endpoints – Change from baseline

CGIS	Clinician Global Impression Severity
CGIC	Clinician Global Impression of Change
PGIS	Patient Global Impression Severity
PGIC	Patient Global Impression of Change
PROM Cog	Patient Reported Perceived Cognition
PGIS-Think	Patient Global Impression Severity Think Clearly
PGIC-Think	Patient Global Impression Change to Think Clearly
PCS	Physical Component Score of SF12
MCS	Mental Component Score of SF12
SF12	QoL
PROM Fatigue	Patient Reported Fatigue Symptoms
DSQPEM	Patient Reported Post-exertional malaise
PGIS-Fatigue	Patient Reported Impression Severity Fatigue
PGIC-Fatigue	Patient Global Impression Change fatigue
PROM Sleep	Patient Reported Sleep Disturbance
Detection S.D	Cognitive Test for Psychomotor Function
Cog Global S.D	Global Cognition score
Identification Attent	Cognitive Test for Attention
IDST Proc Speed	Cognitive Test for Processing Speed
ISLT Verbal learn	Cognitive Test for Verbal Learning
ISLT-R Verbal Mem	Cognitive Test for Verbal Memory
Sust attention	Cognitive Test for Sustained Attention

BIV201 in Cirrhosis with Ascites

BIV201 Disease Target: Ascites

- Fluid build-up in the abdomen of patients with advanced cirrhosis and cannot be controlled with diuretics
- No approved pharmacological treatments – large unmet need
- Standard of care is repeated mechanical draining by paracentesis
 - Withdrawal of 5–10L of ascites fluid (on average) from abdomen using a large bore needle
 - Provides a few days of symptomatic relief
- Mortality > 40% in 12 months¹
- Estimated \$1.6 billion addressable US market with 20,000² targeted patients



Very poor quality of life
Not typically hospitalized

Prefilled Syringe with Patent-Protected Liquid Formulation

BIV201*	Our liquid formulation of terlipressin. Patent protection in the US, Japan, India, China, Chile to date
Accurate dosing	Eliminates mixing minute quantities of powder terlipressin that could result in medication errors or sterility loss
Enhanced convenience	Simply inject fluid into the saline bag and attach to pump

BIV201 Vials & Prefilled Syringe
Stable for 24+months at room temp.



Needle or
Connector



50 mL bag of saline for
insertion into pump

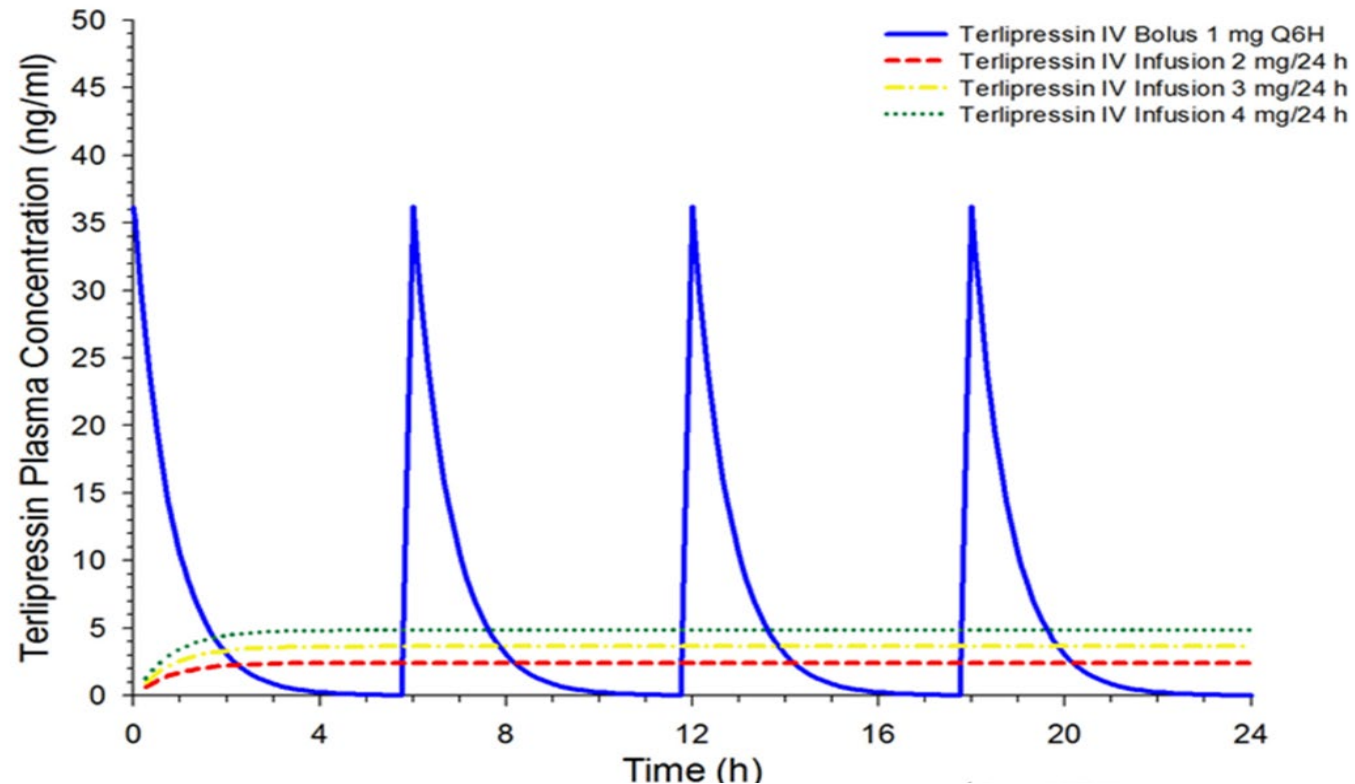


Portable pump
Carried in small satchel



Phase 2A demonstrated safety by avoiding peaks and troughs of IV Bolus

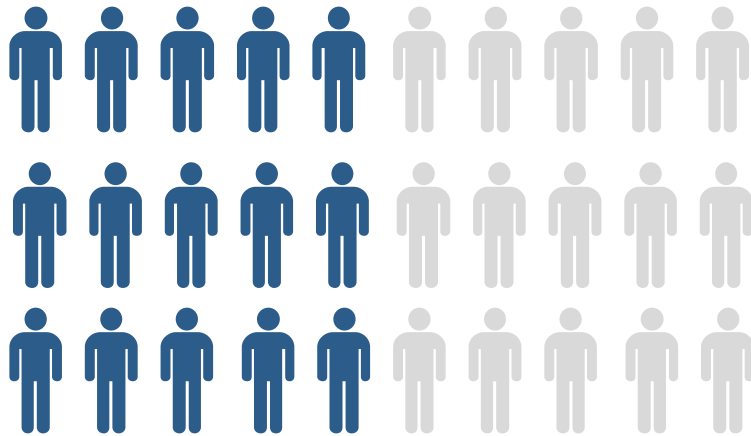
Comparison of the PK Profile of Terlipressin Administered by Continuous Infusion or Intermittent IV Boluses



Stimulated terlipressin plasma concentrations following IV bolus of 1 mg every 6 hours [Lucassin, AusPAR-cer 2011] and IV continuous infusions of 2 mg, 3 mg, or 4 mg per day

Source: Fischer et al., BioVie poster presentation at AASLD 2019.

BIV201 Phase 2b trial

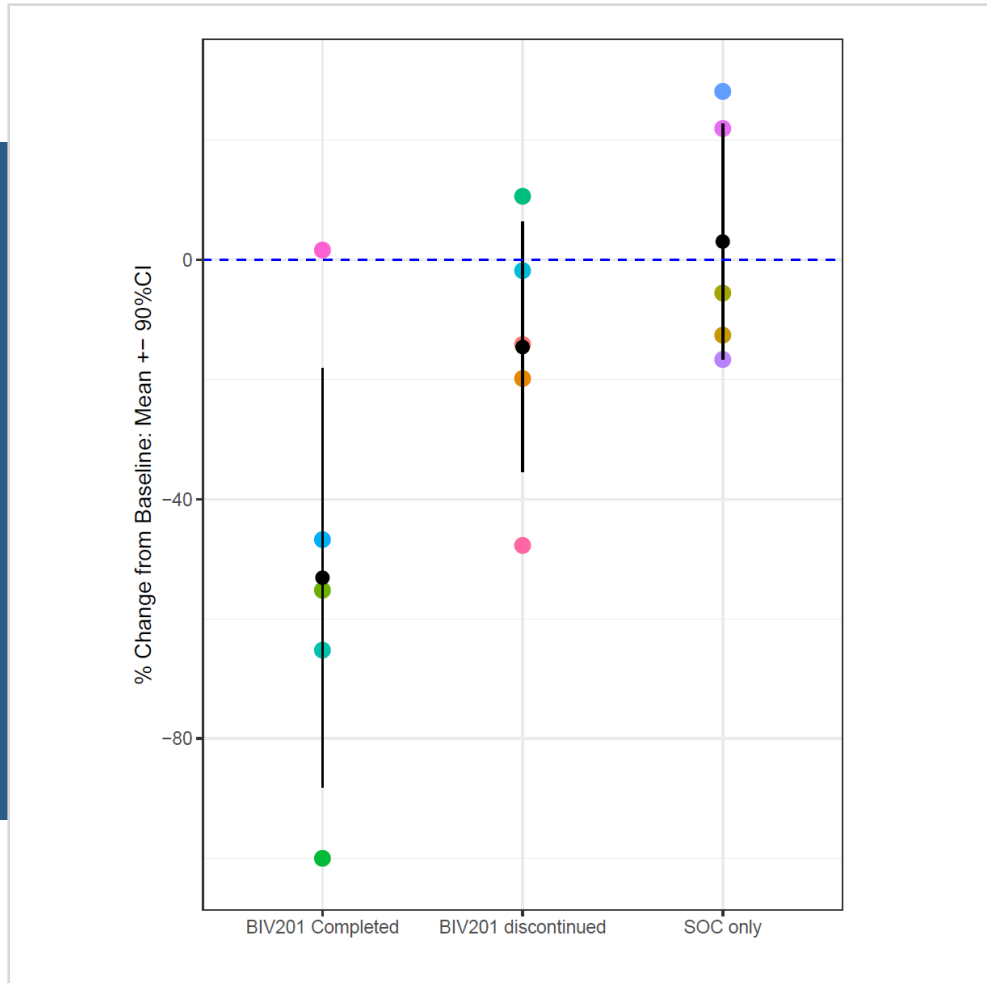


Originally targeted 30 patients randomized 2:1

Paused enrollment based on encouraging data from the first 15 patients informing next steps

- 10 randomized to BIV201; 5 randomized to standard of care
- 5 completed 2 X 28-day cycles
- 5 discontinued treatment during or at end of Cycle 1

Change in ascites volume 28d pre- vs post-treatment



53% reduction in ascites volume among patient completing BIV201 treatment

15% reduction among patients who started but did not complete treatment

3.1% increase for SOC patients

$p < 0.001$

Agreement reached with FDA on Phase 3 design: trial in participants with decompensated liver cirrhosis and ascites who have experienced a recent acute kidney injury

A Phase 3 randomized, controlled, open-label study evaluating the efficacy and safety of BIV201, continuous terlipressin infusion, in addition to standard of care compared to standard of care alone in reducing decompensation events, adverse outcomes, and/or disease progression in adult participants with cirrhosis and ascites who have experienced a recent acute kidney injury

- Male and female ≥ 18 years old diagnosed with cirrhosis and ascites
- Recent (within 1 month) recovery from an AKI
- Randomized (1:1) to 3 months treatment with BIV201 (continuous intravenous infusion of terlipressin acetate) or SOC with 3 months follow-up for adverse events and outcomes and 6 months additional follow-up for major events from medical records. About 200 randomized participants to reach 80% power. 30-40 clinical sites (global)

- **Primary endpoint**

Incidence of participants in the BIV201 plus SOC arm versus the SOC arm alone reaching any one of the four components of the composite endpoint during the 3 months of treatment:

1) All-cause mortality ; 2) Hepatic decompensation event defined as any of the following: Portal hypertension-related GI bleeding including gastro-esophageal variceal bleeding grade ≥ 2 , Documented spontaneous bacterial peritonitis, AKI stage 2, including HRS-AKI, Overt hepatic encephalopathy requiring treatment or hospitalization (West Haven grade ≥ 2), Hepatic hydrothorax ; 3) Liver transplant due to hepatic disease progression defined by an increase in MELD 3.0 score of ≥ 5 points above study entry baseline at time of liver transplant ; 4) Requirement for RRT

- **Secondary endpoints**

- Time to/Incidence of any primary outcome event at 3 mos and 6 mos
- Change in total number of therapeutic paracenteses

Incidence of Major Adverse Kidney Events at 3 and 6 months

Health related quality of life

SOC = Standard of Care; SBP = spontaneous bacterial peritonitis; HE = hepatic encephalopathy;
HRS = hepatorenal; GI = gastrointestinal; AKI = Acute Kidney Injury

Commercial potential in US market alone*

Alzheimer's

\$30B

Annual sales for every
1 million people treated

15% market penetration

\$30K/year much lower all-in cost vs.
competition

Parkinson's

\$3B

Annual sales for every
100,000 people treated

10% market penetration

\$30K/year

Ascites

\$1.6B

US peak sales

45% market penetration

\$45K/year

2026 launch

2032 peak sales

Leadership Team

Deep expertise provides a strong foundation for success



Cuong Do, President & Chief Executive Officer

30+ years in biopharma & technology
President, Samsung Global Strategy Group
Chief Strategy Officer for Merck, Senior partner at McKinsey & Company



Chris Reading, PhD, Neurodegenerative Disease Program

40+ years in biopharma
Chief Scientific Officer, Hollis-Eden Pharmaceuticals
VP of Product and Process Dev. for Systemix
U Texas Dept. of Tumor Biology



David Morse, Chief Regulatory Officer

35 years experience Regulatory Affairs and multi-region product development strategy
Former VP with two top-5 international CRO's
Former Associate Director CDER, FDA



J. Wendy Kim, Chief Financial Officer

35 years in finance/ accounting
As CFO managed corporate finance and operations groups
Closed M&A transactions and secured financings
Combined 22 years at KPMG and BDO LLP



Joseph Palumbo, MD, EVP, R&D and Chief Medical Officer

30+ years treating patients; 25+ years in biopharma
CMO, Zynerva
Global Head of Medical Science & Translational Research, Global Head & Psychiatry Franchise Medical Leader, J&J



Penelope Markham, Liver Cirrhosis Program

25 years in biopharma drug development
Lead Scientist Terlipressin (LATPharma/ BioVie 11 years)
Head Research Biology Protez Pharma
Co-founder/Director of Research Influx Inc.



Clarence Ahlem, Operations

35+ years in biopharma
Vice President, Product Development Harbor Therapeutics
Director, Product Development, Hollis-Eden Pharmaceuticals
US San Diego

Capitalization Table

March 31, 2026		
Common shares outstanding		7,541,539
Warrants (WAEP \$5.56)		8,282,037
Options (WAEP \$7.62)		2,785,363
Restricted Stock units		1,099
Fully dilluted shares outstanding		18,610,038
Market Cap (March 31, 2026 close price \$1.40 /share)	\$	10,558,155

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- Reduced inflammation and the associated insulin resistance
- Improved motor control and “morning on” symptoms in Parkinson’s disease (PD)
- Improved cognition and function, lowered amyloid β and p-tau levels, and improved brain imaging scans in Alzheimer’s Disease (AD)
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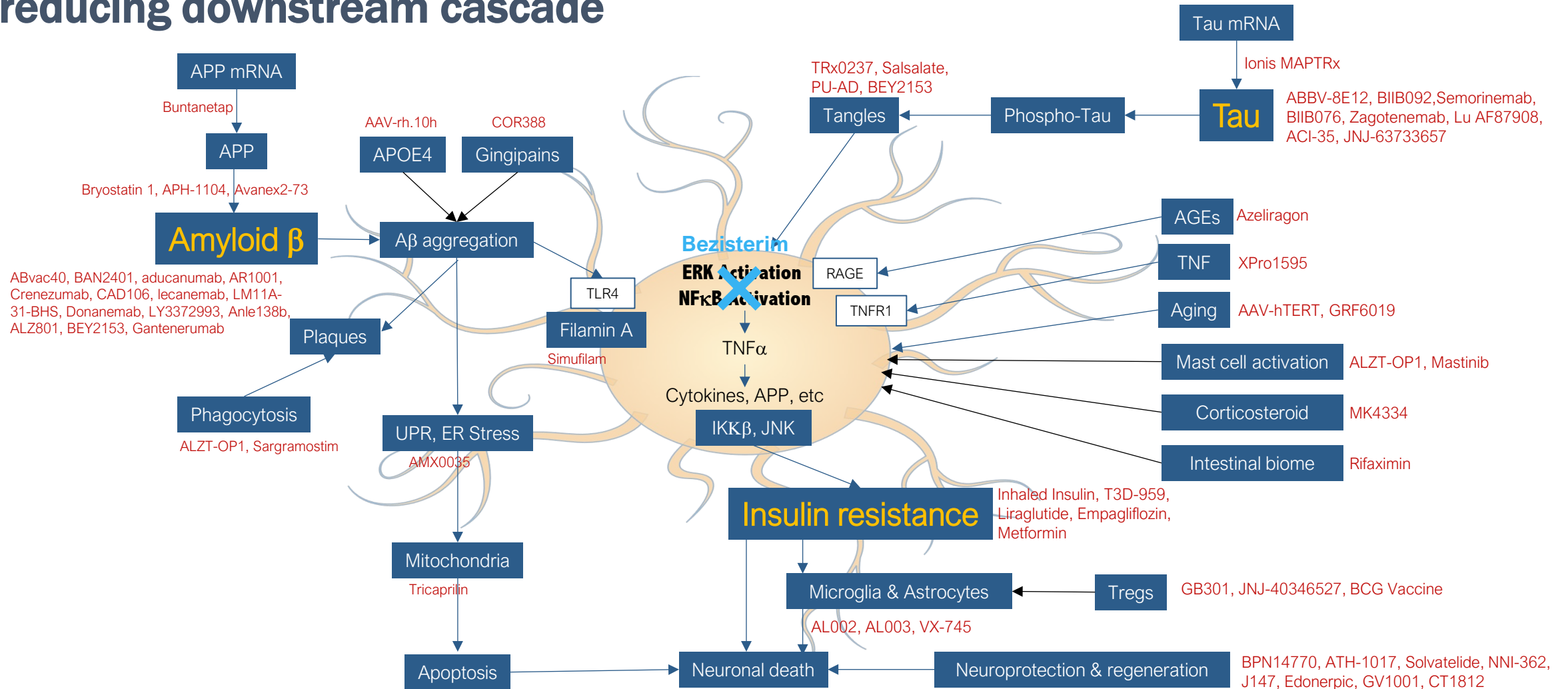
Our priorities:

- Enroll patients for the opened Parkinson’s Phase 2b trial and Long Covid exploratory Phase 2
- Launch next Alzheimer’s Phase 3 trial using new once-daily formulation of bezisterim upon securing funding
- Launch ascites Phase 3 when partner identified

Thank You

Bezisterim in Alzheimer's Disease

Bezisterim modulates inflammation at the central hub, thereby potentially reducing downstream cascade



NM101 Phase 3 trial in Mild to Moderate Alzheimer's

A Phase 3, Double-blind, Randomized, Placebo-controlled, Parallel Group, Multicenter Study of bezisterim. Enrolled 439 Patients who have Mild to Moderate AD

- Pivotal study for Alzheimer's disease. Two weeks each of 5 mg and 10 mg BID dose titration followed by 26 weeks of 20 mg twice daily vs. placebo; 1:1 randomization; 80% power
- Diagnosed with probable AD and without evidence of a vascular contribution. Mild to moderate disease. CDR 1-2. MMSE 14-24.
- 60-85 years old, males and females
- Randomization stratified by MMSE and Homeostatic Model Assessment 2 Insulin Resistance (HOMA2)
- **Co-primary endpoints**
 - Mean change from Baseline to Week 30 in Dementia Rating-Sum of Boxes (CDR-SB) comparing the bezisterim group to the placebo group
 - Mean change from Baseline to Week 30 in the twelve-question form of the Alzheimer's Disease Assessment Scale-Cognitive (ADAS-Cog 12) comparing the bezisterim group to the placebo group
- **Secondary endpoints**
 - ADCS-ADL (functional), Alzheimer's Disease Cooperative Study-Clinical Global Impression of Change (ADCS-CGIC), ADCOMS (4 Alzheimer's Disease Assessment Scale-cognitive subscale items, 2 Mini-Mental State Examination items, and all 6 Clinical Dementia Rating-Sum of Boxes items), NPI-12 (care-giver rating of behavioral changes), MMSE, CDR
 - Glycemic control: HOMA2, Mean Amplitude of Glycemic Excursion (MAGE) using continuous glucose monitoring, fructosamine levels, post-prandial glucose and fasting blood glucose vs time.
 - MRI total hippocampus volume change, baseline to end of treatment in a subset of active and placebo subjects
 - Target engagement assessed in a small subset of active and placebo subjects using PET to quantify cortical glucose utilization

Trial Summary

- Bezisterim appears to be biologically active
- Cognitive, functional, biomarker efficacy signal suggest that bezisterim:
 - Has a treatment advantage equal to or greater than results reported from clinical trials from approved monoclonal antibody treatments;
 - Associated with a benign safety profile
- Unanticipated exclusion of 258 patients from 15 sites due to deviations led to study being underpowered

Week 30 Suggest bezisterim Advantage vs. Placebo is Comparable to or Better than Results Reported from Clinical Trials by Approved Medications

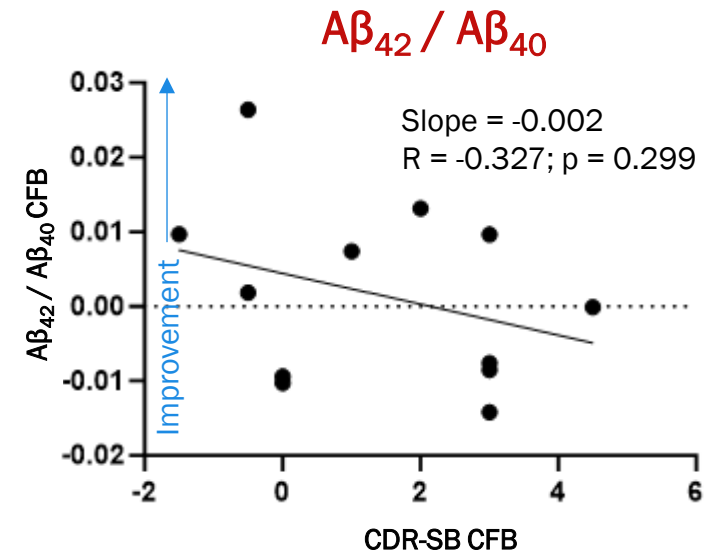
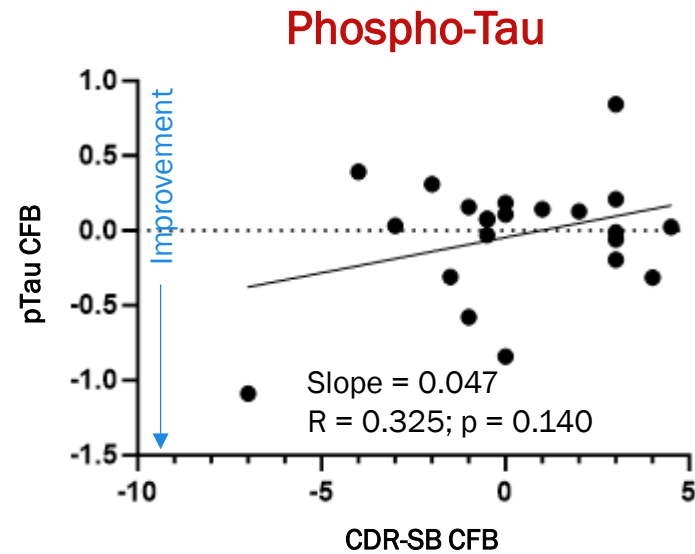
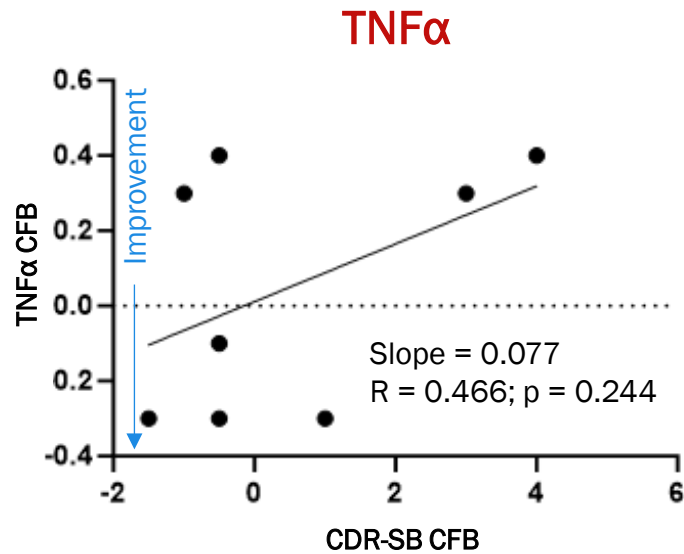
Change from baseline

	Placebo Decline	Bezisterim	Bezisterim vs. Placebo	Comparator (18 mos)
CDR-SB (lower is improvement)	+1.39 (p=0.0125; n=26)	+0.44 (p=0.4522; n=24)	-0.95 (68%) (p=0.2278)	-0.45 (27%) ¹ -0.39 (22%) ²
ADAS-Cog12 (lower is improvement)	+3.64 (p=0.0545; n=23)	+2.70 (p=0.1618; n=24)	-0.94 (26%) (p=0.7212)	-1.44 (25%) ¹ -1.40 (27%) ²
MMSE (higher is improvement)	-2.54 (p=0.0007; n=26)	-1.52 (p=0.0547; n=24)	+1.02 (40%) (p=0.3181)	+0.6 (18%) ²
ADCS-ADL (higher is improvement)	-6.54 (p<0.0001; n=27)	-3.46 (p=0.0435; n=24)	+3.08 (47%) (p=0.1620)	+2.0 (36%) ¹
ADCS-CGIC (lower is improvement)	+0.31 (p=0.2733; n=26)	-0.12 (p=0.6951; n=24)	-0.43 (139%) (p=0.2866)	
ADCOMS (lower is improvement)	+0.11 (p=0.0358; n=22)	+0.09 (p=0.1094; n=24)	-0.03 (27%) (p=0.7063)	-0.05 (23%) ¹

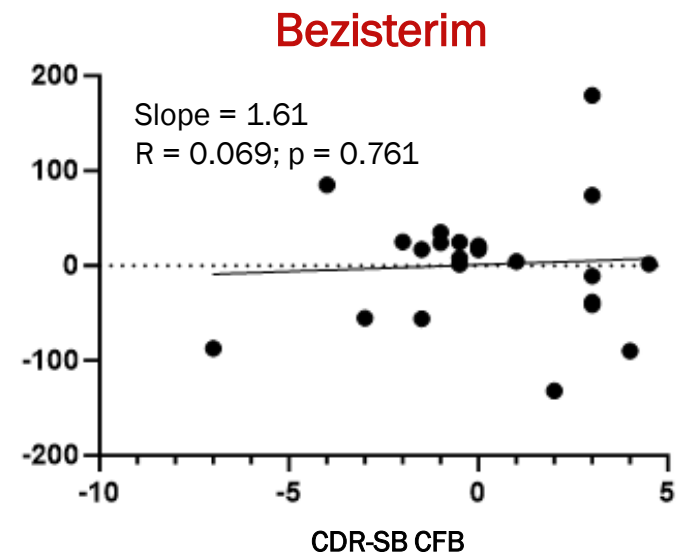
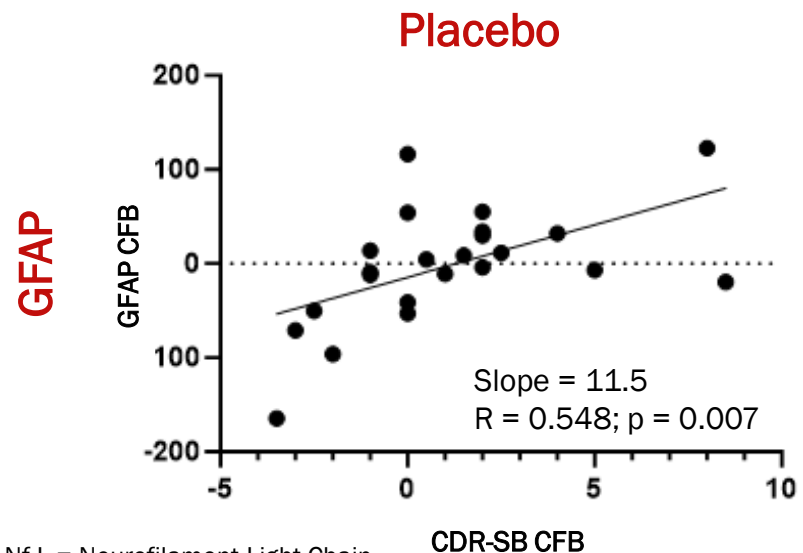
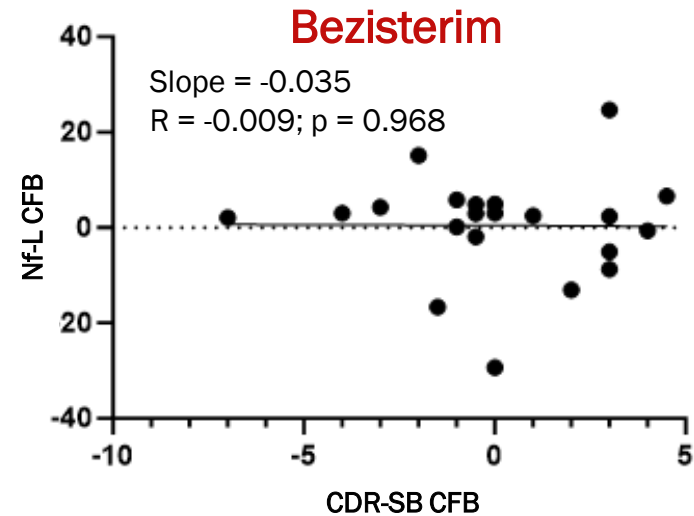
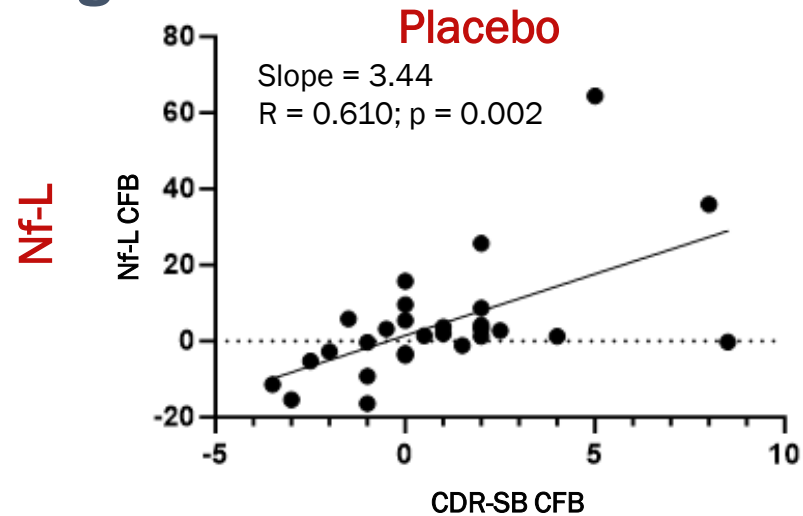
¹ Lecanemab after 18 months; van Dyck et al. *N Engl J Med* 2023;388:9-2

² Aducanumab after 18 months; Haeberlein et al. *J Prev Alz Dis* 2022;2(9):197-210

Bezisterim-treated Patients' Changes in CDR-SB Appears Correlated with Key Biomarkers



Bezisterim Appears to Decrease the Neuroinflammatory Processes that Link Nf-L and GFAP to Cognitive Decline



Safety Profile

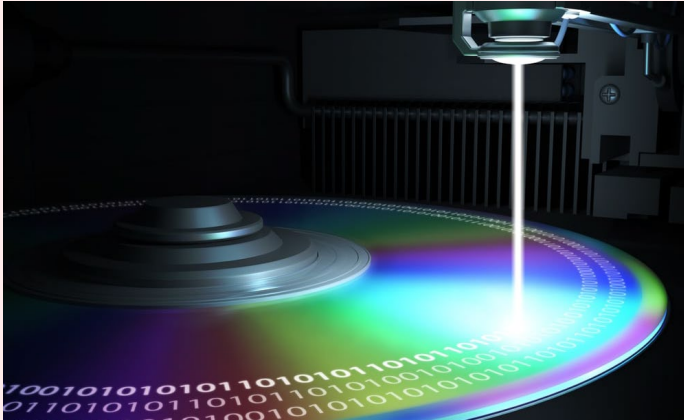
	NE3107	Placebo	Total
COVID-19	9.5%	17.6%	13.2%
Urinary tract infection	7.1%	8.8%	7.9%
Blood thyroid stimulating hormone increased	7.1%	2.9%	5.3%
Fall	2.4%	8.8%	5.3%
Headache	9.5%	0.0%	5.3%
Diarrhoea	4.8%	2.9%	3.9%
Dizziness	2.4%	5.9%	3.9%
Hypertension	2.4%	5.9%	3.9%
Nausea	4.8%	2.9%	3.9%
Pneumonia	4.8%	2.9%	3.9%
Vomiting	2.4%	5.9%	3.9%
Blood testosterone decreased	0.0%	5.9%	2.6%
Gastroenteritis viral	0.0%	5.9%	2.6%
Nasopharyngitis	4.8%	0.0%	2.6%
Rash	0.0%	5.9%	2.6%
Thyroxine decreased	2.4%	2.9%	2.6%
Tri-iodothyronine decreased	2.4%	2.9%	2.6%
Abdominal pain	0.0%	2.9%	1.3%
Abdominal pain upper	0.0%	2.9%	1.3%
Accelerated idioventricular rhythm	2.4%	0.0%	1.3%
Agitation	0.0%	2.9%	1.3%
Aortic valve replacement	2.4%	0.0%	1.3%

	NE3107	Placebo	Total
Atrioventricular block first degree	2.4%	0.0%	1.3%
Bile duct stone	0.0%	2.9%	1.3%
Blood lactate dehydrogenase abnormal	0.0%	2.9%	1.3%
Blood prolactin decreased	2.4%	0.0%	1.3%
Blood prolactin increased	2.4%	0.0%	1.3%
Blood sodium abnormal	0.0%	2.9%	1.3%
Blood sodium increased	0.0%	2.9%	1.3%
Blood thyroid stimulating hormone decreased	0.0%	2.9%	1.3%
Bronchitis	0.0%	2.9%	1.3%
Calculus bladder	2.4%	0.0%	1.3%
Cholelithiasis	2.4%	0.0%	1.3%
Cough	0.0%	2.9%	1.3%
Delirium	0.0%	2.9%	1.3%
Dementia Alzheimer's type	2.4%	0.0%	1.3%
Dermatitis	2.4%	0.0%	1.3%
Dysphagia	2.4%	0.0%	1.3%
Dysuria	0.0%	2.9%	1.3%
Electrocardiogram abnormal	2.4%	0.0%	1.3%
Eosinophil count increased	2.4%	0.0%	1.3%
Eustachian tube dysfunction	2.4%	0.0%	1.3%
Hordeolum	0.0%	2.9%	1.3%
Hyperkalaemia	2.4%	0.0%	1.3%

	NE3107	Placebo	Total
Hypothyroidism	2.4%	0.0%	1.3%
Hypoxia	0.0%	2.9%	1.3%
Incontinence	0.0%	2.9%	1.3%
Increased appetite	2.4%	0.0%	1.3%
Influenza	0.0%	2.9%	1.3%
Insomnia	2.4%	0.0%	1.3%
International normalised ratio increased	0.0%	2.9%	1.3%
Lethargy	2.4%	0.0%	1.3%
Lipase increased	0.0%	2.9%	1.3%
Muscle spasms	2.4%	0.0%	1.3%
Nephrolithiasis	0.0%	2.9%	1.3%
Nightmare	0.0%	2.9%	1.3%
Obsessive-compulsive disorder	2.4%	0.0%	1.3%
Oesophageal food impaction	0.0%	2.9%	1.3%
Optic ischaemic neuropathy	2.4%	0.0%	1.3%
Orthostatic hypotension	2.4%	0.0%	1.3%
Papilloedema	2.4%	0.0%	1.3%
Paranasal sinus discomfort	0.0%	2.9%	1.3%
Patient elopement	0.0%	2.9%	1.3%
Pelvic fracture	0.0%	2.9%	1.3%
Pharyngitis streptococcal	2.4%	0.0%	1.3%

Bezisterim in Longevity

Clean DVD leads to clear
picture and sound



Scratches & smudges lead
to skips and blurs



Impact of wear & tear on a laser's ability to decode DVDs

Quality of picture is dependent
on the laser's ability to clearly
decode the disk ...

The same thing happens in our
body

DNA methylation

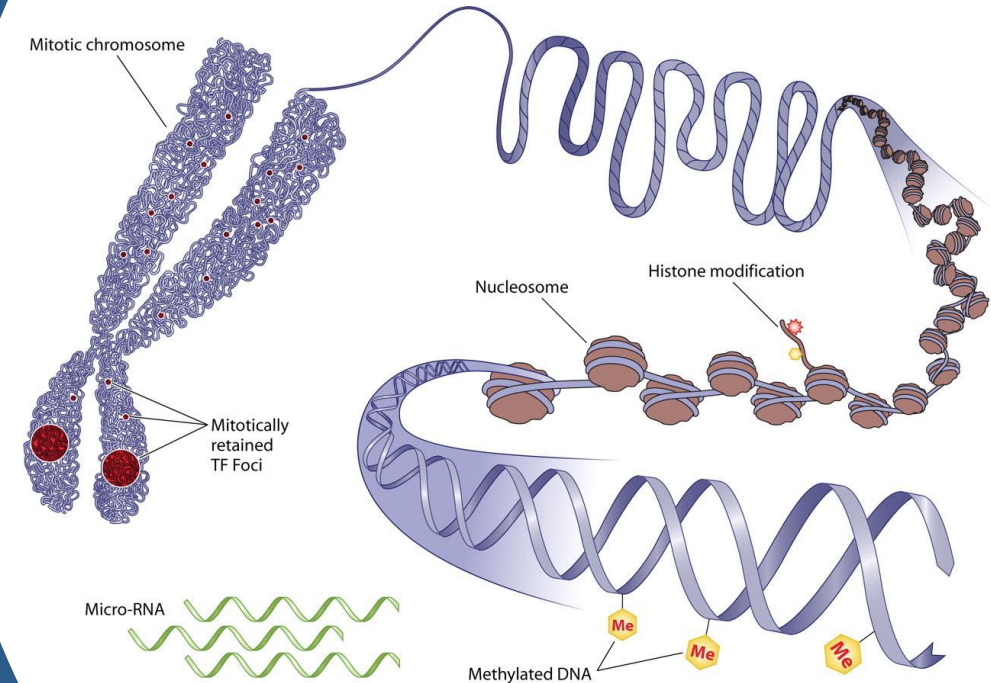
DNA methylation happens when methyl groups are added to our DNA

- DNA methyltransferases add methyl groups to DNA
- Functionally the equivalent of scratches and smudges on a DVD surface
- The methyl groups interfere with RNA polymerase's ability to decode DNA

DNA methylation may happen where a cytosine is positioned next to guanine and is separated by a phosphate group (CpG)

- 28 million CpGs in genome

Hypermethylation of DNA is associated with many disease conditions



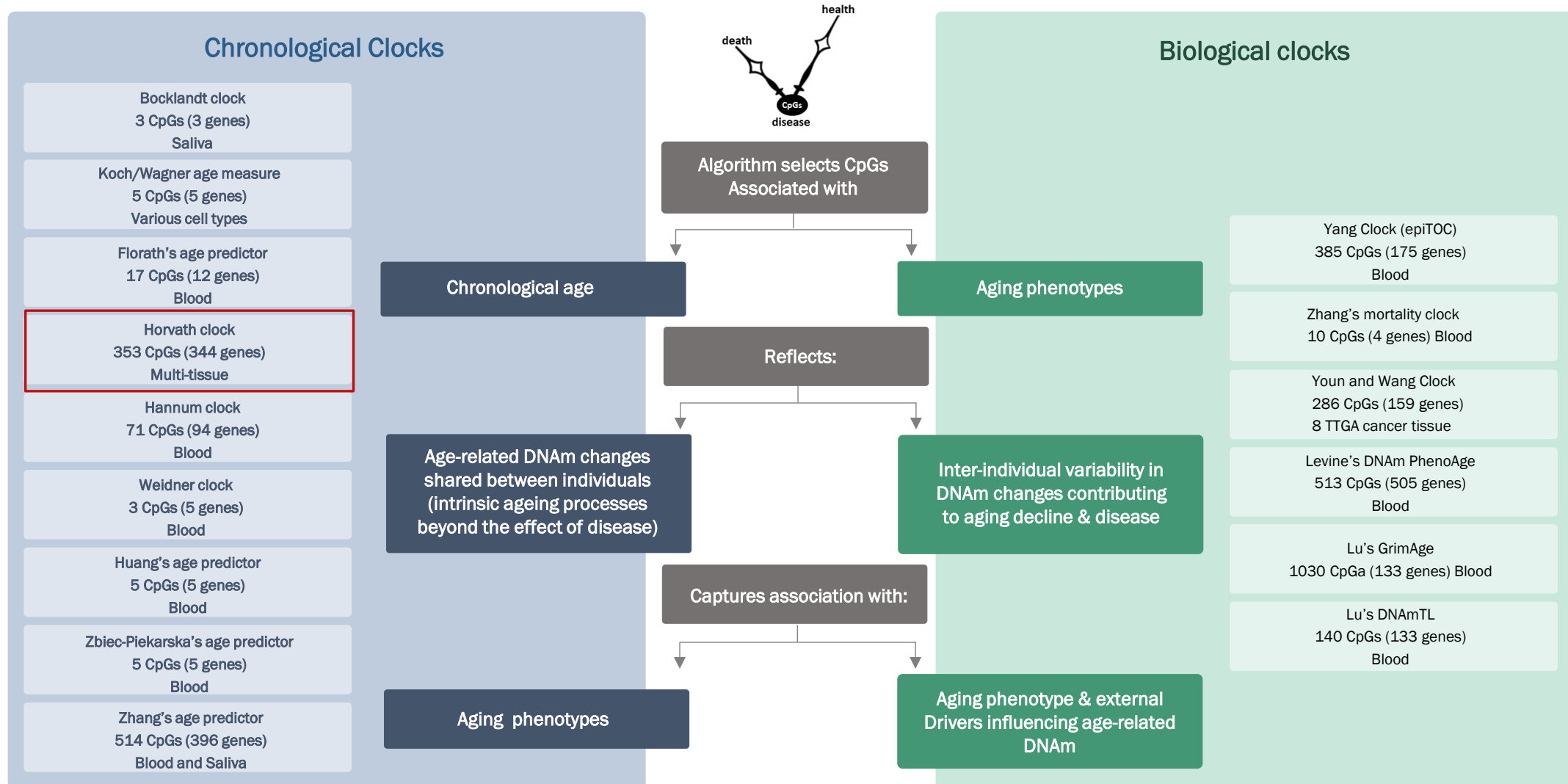
Observations about DNA methylation

- DNA methylation increases as we age
- DNA methylation can be affected by behavioral (diet, exercise) and environmental factors
- DNA hypermethylation is associated with a large number of disease conditions, including various forms of cancers, age-related cognitive impairment and dementia, Parkinson's disease, cardiovascular disease, COPD and respiratory disease, chronic kidney disease, inflammatory bowel disease, sepsis, and many others*
- Inflammation has been shown to be a driver of hypermethylation of DNA**
- Extent of DNA methylation can be measured by various “clocks”

* Wang Z Nucleic Acids Research, 2020, Vol. 48, No. 5; Sugden K Neurology 2022;99:e1402-e1413; Tang X DOI: 10.1002/mds.29157; Tabaeia S Artificial Cells, Nanomedicine, and Biotechnology, 47:1, 2031-2041; Qiu W Am J Respir Crit Care Med Vol 185, Iss. 4, pp 373–381, Feb 15, 2012; Rysz C Int. J. Mol. Sci. 2022, 23(13), 7108; Krawczyk J Mucosal Immunology volume 9, pages 647–658 (2016); Rump K Sci Rep 9, 18511 (2019)

** Stenvinkel P doi: 10.1111/j.1365-2796.2007.01777.x

DNA methylation “clocks” measure extent of aging and biological function



Bezisterim-treated subjects showed a lower biological age than placebo subjects

Age Deceleration¹ is used by longevity researchers² to measure the difference between a person's biological age and the actual chronological age.

